

Physical Therapy Evaluation

Patient	Brandon Wright
Date of Evaluation	January 16, 2026
Therapist	Marit Olsen, DPT
Referring Provider	Amy Sorensen, MD
Sport	Cross-country ski racing and run training

Subjective

Athlete reports recurring aching along the medial border of the right tibia that flares during high-volume running weeks and eases on snow. No night pain, no numbness. Pain rated 3 of 10 at rest, 5 of 10 at the end of a long run. History of the same since high school track.

Objective

Finding	Result
Palpation, distal medial right tibia	Tender over a 6 cm band, no focal bone pain
Single-leg hop test	Negative for stress fracture signs
Ankle dorsiflexion range	Reduced on right, 8 degrees vs 12 left
Calf strength	4+/5 right, mild deficit
Running gait	Slight overstride, increased tibial loading right

Assessment

Chronic right medial tibial stress syndrome, consistent with prior episodes. No clinical indicators of tibial stress fracture at this time. Contributing factors include limited ankle dorsiflexion and an overstriding running pattern.

Plan

- Calf and tibialis posterior eccentric loading, daily.
- Ankle mobility work to restore dorsiflexion.
- Increase running cadence by 5 to 8 percent to reduce overstride.
- Keep volume on soft surfaces and cross-train on snow and roller skis during flares.
- Follow up in 4 to 6 weeks or sooner if focal pain develops.